جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : يسرا صالح عبدالحكم رضوان
المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** المعهد العالي الصناعي
رقم البطاقة : 28107042500687 **رقم التليفون :** 01114521478 **تاريخ تسجيل الإستمارة :** 14-10-2026
العمر(تاريخ الميلاد) : 04-07-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: يسرا صالح عبدالحكم رضوان
National ID: 28107042500687

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

يسرا
صالح عبدالحكم رضوان
٢ ش قليب حنا-المعهدالعالى الصناعى
عين شمس - القاهره

٢٨١٠٧٠٤٢٥٠٠٦٨٧

١٩٨٦/٠٧

LG6382299

فتح ملف
د/ فريد عبد الله

٢٠٢٥/٠٧

٢٨١٠٧٠٤٢٥٠٠٦٨٧
حاصله على ليسانس اداب

أنثى مسلمة متزوجة
احمد كمال محفوظ محمد
البطاقة سارية حتى ٢٠٣٢/٠٧/٢٧

١



National ID - Front
[[DOCTYPE_ID_FRONT]]

Visit Number:	10382605867	Client Name:	Walk-in patient
Patient Name:	بسرا صالح عبدالحكم	Referred by Prof:	مصطفى عباس
Age / Sex:	45 year / Female	Exam Date:	15 Aug 2026
Patient ID:	2140316365	Printed:	16 Aug 2026 05:52:49 PM

MRI

DYNAMIC MR MAMMOGRAPHY

CLINICAL HISTORY: ***Follow up post neoadjuvant chemotherapy for pathologically-proven left breast malignancy-Benign right breast mass on sonomammography.***

TECHNIQUE:

USING 3.0 TESLA MRI and DEDICATED BREAST COIL:

Axial T1, TIRM and T2 WIs.
Sagittal T2 WIs.
Five axial dynamic contrast-enhanced sequences, after bolus injection of 0.1 mmol/kg of contrast, using 1.6 mm slice thickness.
MIP images.
Axial Diffusion-weighted images.
Time/intensity enhancement curves for enhancing lesions.

COMPARISON: *Sonomammography done on 04-02-2026.*

FINDINGS:

- *Moderate background parenchymal enhancement of both breasts (ACR c).*

RIGHT BREAST:

Scan Result

Mammogram
[[DOCTYPE MAMMOGRAM]]

Visit Number:	10382605867	Client Name:	Walk-in patient
Patient Name:	يسرا صالح عبدالحكم	Referred by Prof:	مصطفى عباس
Age / Sex:	45 year / Female	Exam Date:	15 Aug 2026
Patient ID:	2140316365	Printed:	16 Aug 2026 05:52:49 PM

MRI

- *An oval-shaped, well-circumscribed mass is noted at 9 o'clock, 2.0 cm from the nipple, measuring 1.5 x 1.2 cm. It exhibits intermediate T1 and T2 signals, with rather homogeneous enhancement, demonstrating type 1 time/intensity curve (mild peak, followed by progressively increasing contrast uptake).*



- *No pathological axillary, internal mammary or intra-mammary lymph nodes detected.*

LEFT BREAST:

- *A non-circumscribed, spiculated mass is noted at 4 and 5 o'clocks, involving the posterior depth of the breast parenchyma, located 9.5 cm from the nipple, measuring 3.0 x 3.0 x 2.0 cm, in craniocaudal, anteroposterior and transverse dimensions, respectively.*



- *On the dynamic post contrast study, the fore mentioned mass demonstrates heterogeneous enhancement, with type 1 time/intensity curve (mild peak, followed by progressively increasing contrast uptake).*
- *The fore mentioned mass is surrounded by an area of architectural distortion, containing enhancing tentacles extending from the*

Scan Result

Visit Number:	10382605867	Client Name:	Walk-in patient
Patient Name:	يسرا صالح عبدالحكم	Referred by Prof:	مصطفى عباس
Age / Sex:	45 year / Female	Exam Date:	15 Aug 2026
Patient ID:	2140316365	Printed:	16 Aug 2026 05:52:49 PM

MRI

mass into the surrounding breast parenchyma, overall (including the mass) measuring 4.5 x 6.5 x 3.0 cm, in craniocaudal, anteroposterior and transverse dimensions, respectively, reaching the overlying skin.

- *An enhancing, mildly dilated duct is seen at 6 o'clock, in the anterior and middle depths of the breast parenchyma, reaching the nipple, measuring 0.6 cm in diameter (intra ductal extension).*
- *Located 3.2 cm from the superior edge of the mass, a 1.0 x 0.6 cm satellite is noted at 2-3 o'clock, in the posterior depth of the breast parenchyma, located 9.8 cm from the nipple. Linear, non-mass enhancement (NME) is seen 1.5 cm anterior to the satellite, measuring 1.4 x 0.2 cm.*
- *A pathological axillary level 1 lymph node is noted, measuring 1.0 x 0.5 cm.*
- *No pathological axillary levels 2 or 3, internal mammary or intra-mammary lymph nodes detected.*

OPINION:

- ***Follow up post neoadjuvant chemotherapy for pathologically-proven left breast malignancy (BIRADS VI), with partial response, as described.***
- ***Benign-looking, right breast mass, likely fibro adenoma (BIRADS III).***

Scan Result

M.R.I
[[DOCTYPE MRI]]

البـــرج
معامل و سكان

Visit No.:	10382605867	**Patient Name:**	الأستاذة / يسرا صالح عبدالحكم	**Patient ID:**	2140316365
Visit Date:	15 Aug 2026	**Branch :**	Alborg Scan - New Cairo	**Age:**	45 year
Receipt No.:	8552	**Ref. Doctor:**	مصطفى عباس	**Result Date:**	
Payment Date:	15-08-2026 14:00:00	**Customer:**	Normal Patient	**Insurance No**	

Item Type	Item Name	Value
Service	Contrast MRI- 10 ml Walk in	1650
Study	MRI breast	3955

Current Receipt Amount:	5605	**Total Amount:**	5605
Payment User Name:	essa hosny hassan	**Discount:**	0
Payment Date Time:	8/15/2026 2:00:05 PM	**Final Amount:**	5605
		Total Received:	5605
		Balance Amount:	0

اجمالي المبلغ المدفوع : 5605 جنيهاً

الأستلام اليوم التالى للفحص الساعة 9 مساء (عدا الجمعة) - نتائج القلب بعد 48 ساعة خلال مواعيد العمل الرسمية

User ID : bg_2140316365
Password : CAGBJAFY

يتم تسوية حساب التحويلات خلال ثلاثة أيام من هذا الإيصال وذلك كحد أقصى يسقط بعده حق المطالبة باسترداد المبلغ وفي حالة التسويه يلتزم العميل بإحضار التالي (خطاب التحويل - صورة البطاقة الشخصية للمريض او للوالد في حالة الطفل - هذا الإيصال)

- يتم الاحتفاظ بصور أفلام الأشعة بحد أقصى ستة أشهر

لإستفساراتكم وخدمة الزيارة المنزلية وتوصيل النتائج اتصل بخدمة العملاء من اي تليفون على - **19911**

أستلام النتائج من اى فرع من فروع معامل البرج او من خلال الموقع الالكترونى مجانا **https://www.alborgscan.com/**

كما يمكنكم أستلام النتائج عن طريق تحميل تطبيق - (البرج سكان) يمكن تحميل صورة الأشعة عن طريق خدمة (الواتس اب)

العنوان : ميديكال بارك برايمير قطعة 165 القطاع الاول

تليفون : 19911

استلامكم لهذا الايصال يعد موافقة من حضراتكم بالسماح لمركز البرج سكان للأشعة باستخدام البيانات الشخصية في الرسائل التذكيرية و في حالة الرفض او العدول عن تلك الموافقة الاتصال ب 19911

Non UNIA Letter
[[DOCTYPE:NON_UNIA_LETTER]]

الكرنك للأشعة والتحاليل
Karnak Scan & Lab

Patient Name: Mrs. Yousra Saleh Abdel Hakeem Radwan
Date: 04/02/ 2026
Ref. By: Prof. Dr. Mohamed Abdel Monaem El Mekkawy

BILATERAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASONOGRAPHY REVEALED:-

CLINICAL SUMMARY:

- 3 weeks history of left breast lump sensation.

FINDINGS: -

- Normal skin thickness and contour with normal nipple/ areola complex and subcutaneous tissues.
- Heterogeneous fibro-glandular element on both sides.
- **The left breast, at the UOQ shows irregular focal asymmetry at the UOQ, with coarse heterogeneous micro calcifications. The left breast shows another group of suspicious micro calcification at the outer central/ retro areolar region. In ultrasound, seen along 3 O'clock orientation (At the anterior axillary line/ site of complaint/ palpable lump) as a hypoechoic ill-defined mass lesion, measuring at least 4.3x3.5x3 cm, with mild internal vascularity. It is seen related to dilated ducts, extending to the nipple (related to the aforementioned retro areolar micro calcifications.**
- **The right breast, at the UOQ, along 10 O'clock orientation, about 2 cm from the nipple and 0.5 cm from the skin shows an oval, circumscribed hypoechoic mass lesion with macro lobulated outlines, measuring about 2x1 cm. no internal vascularity detected.**
- No suspicious solid or cystic lesions noted on the right side.
- No significant retro areolar ducts on the right side.
- No suspicious micro-calcifications seen on the right side
- Bilateral axillary reactive lymph nodes with preserved fatty hila. **One of these nodes on the left axilla (measuring 1.5x0.7 cm, shows focal cortical thickening (4.5mm).**

OPINION: -.

- Left breast UOQ highly suspicious irregular mass lesion, with possible intraductal extension reaching the retro areolar region as well as one indeterminate left axillary lymph node (BIRADS 4c), for trucut needle biopsy.
- Right breast UOQ mass lesions of favorable criteria (? Fibroadenoma (BIRADS 3), for close follow up.



Dr. Mariana Fikry, Msc

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karnakscanrec@gmail.com
٨٣ شارع عين شمس – أمام فرع أورنج – القاهرة
٢٤٩٢٣٧٠٢ – ٢٤٩٩٣١٣٣
٠١٢٢٢٣٥٣٧٢ – ٠١٠٦٥٨٨٣٩٩٠ – ٠١١٥٣٣٨٨٨٨٠
٠١١٤١٩١٧٢٧٩
www.karnakscan.com

Mammogram
[[DOCTYPE MAMMOGRAM]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : يسرا صالح عبد الحكم رضوان **Analysis ID :** 1038266

Age : 44 Y : 7 M : 2 D **Gender :** Female **Conf. Code :** 9855

Referred By Prof. Dr. : مصطفى عباس

Clinical Diagnosis : Left breast mass and left axillary lymph node.

Nature of Specimen : Referred blocks (tru cut biopsies), labeled by name (Done 1+2).

Receiving Date : 2026-03-01 **Patient ID :** 762347

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block then treated by monoclonal antibodies and detection kit (EnVision FLEX) using DAB as chromogen and hematoxylin as counter-stain, using Omnis machine.
The sections were treated against:
- Estrogen receptors - Progesterone receptors
Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.
The sections were treated against:
- Her 2 - Ki67

Results :

90% of tumor cells show STRONGLY POSITIVE Staining for Estrogen receptors
100% of tumor cells show STRONGLY POSITIVE Staining for progesterone receptors.
Sections treated for HER2 revealed NEGATIVE CELL MEMBRANE STAINING.
30% of tumour cells are positive for KI67.

Conclusion :

Referred blocks(tru cut biopsies), TUMOUR CELLS ARE
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8 / 8)
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 8 / 8)
HER 2 NEGATIVE, SCORE 0, WITHOUT CELL MEMBRANE STAINING [NULL].
THE PROLIFERATION INDEX OF Ki67 IS 30 %.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧

Immunohistochemistry
[[DOCTYPE.IHC]]

Dr. saad S. Eissa, M. D.
Dd' Etat C E S Path (Paris), D.A.E. (Paris)
Fellow Johns Hopkins Medical School (USA)
Dr. Mohab S. Eissa, M. D.
M.D. Pathology Oncology
Pathology Consultant

دكتور سعد سيد عيسى
دكتوراة باثولوجى - باريس
زميل جامعة جونز هوبكنز - الأمريكية
دكتور مهاب سعد عيسى
دكتوراة باثولوجيا الاورام
إستشارى باثولوجيا

PATHOLOGY REPORT

Name: YOUSRA SALEH ABDEL HAKIM **Age:** *years*

Clinical picture: Left Breast Cancer, Post Neoadjuvant Chemotherapy

Nature of specimen: Left Wide Local Excision and Axillary Clearance Subjected to Frozen Section Control

Referred by: Prof. Dr. **ASSEM EL BARASHY** **Date:** 29 / 8 / 2026

Gross:

Three specimens were received:

1. *Left wide local excision specimen 12x10 cm covered by skin ellipse 4x1.5 cm. Serial sectioning revealed a fairly defined rubbery whitish area 3x3 cm. The area is located 1 cm from the least superior margin of excision, 2 cm from the inferior, 2.5 cm from the medial, 2 cm from the lateral, and 2 cm from the deep. Margins are subjected to frozen section control: All are negative for tumour extension.*
2. *Extra-retroareolar margin: Rubbery fatty tissue 2.5x2.5 cm, totally submitted. Margin is subjected to frozen section control: Free of tumour.*
3. *Axillary clearance: Fatty tissue piece 9x7 cm entangling twenty lymph nodes, the largest is 1x1 cm.*

Microscopic:

1. *Sections examined revealed areas of fibrohyalinosis and calcifications depicting therapy effect around areas of residual viable mixed invasive duct and lobular carcinoma, grade 2. Lobular carcinoma in-situ and atypical lobular hyperplasia are noted, with overall tumour viability of 30%. No lymphovascular emboli detected. All margins of excision namely the superior, inferior, medial, lateral, and deep are negative for tumour spread.*
2. *Extra retroareolar margin is negative for tumour cells.*
3. *Axillary clearance: Three of the twenty dissected lymph nodes are positive for macrometastasis (3/20) with no extranodal extension.*

Diagnosis:

LEFT BREAST, WIDE LOCAL EXCISION AND AXILLARY CLEARANCE UPON FROZEN SECTION CONTROL: STATUS POST CHEMOTHERAPY FOR MIXED INVASIVE DUCT AND LOBULAR CARCINOMA, GRADE 2, TUMOUR VIABILITY: 30%. RCB CLASS: III. RESECTION MARGINS ARE FREE. AXILLARY LYMPH NODES ARE POSITIVE FOR METASTASIS (3/20). STAGE: [ypT2N1].

(Code: 595 / 8 / 2026) Date of issue: 31 / 8 / 2026

معمل ٦-١٢ بطرس غالى متفرع من إبراهيم اللقانى ميدان روكسى - مصر الجديدة - الدور الخامس
معمل : ٢٤٥٠٥٧٢٤ - ٢٨١٤٠١٨
منتب : ٢٨١٠٨١٠٥
E-Mail : drsaadeissa@hotmail.com

Pathology Report
[[DOCTYPE PATHOLOGY_REPORT]]

مركز بيتا كير لعلاج الأورام

ONCOLOGY CENTER
BETA CARE
BE TOGETHER AGAINST CANCER

تقرير طبي

ONCOLOGY TEAM

DR. WAHEED ABD ELMENAM DM
PROF. OF CLINICAL ONCOLOGY AIN SHAMS UNIVERSITY

DR.MOSTAFA ABBAS MD
CLINICAL ONCOLOGY CONSULTANT (CEO)

DR.RASHA SHALTOT MD
MEDICAL ONCOLOGY CONSULTANT

DR. OSAMA ELMENSHAWY
CLINICAL ONCOLOGY R

DR. AHMED EMAD
CLINICAL ONCOLOGY R

HAEMATOLOGY UNIT

DR. MOHAMED ELSAID MD
HEAMATO-ONCOLOGY CONSULTANT

DR. AHMED HELMY MD
HEAMATO-ONCOLOGY CONSULTANT

RADIOLOGY UNIT

PROF. AMR MOHAMED MD

DR. RAGAB SAMIR MSC

DR. MOHAMED ELMENNAWY

DR. DOAA ELTAYEB

NUTRITION UNIT

DR. ENGY MAGDY MSC

NEUROPSYCHIATRY UNIT

DR. LAMIS ELDABKY
PSYCHOLOGIST

CLINICAL PHARMACY UNIT

DR. AMIRA YOUSRY MOHAMED ALI

DR. ISLAM MOHAMED IBRAHIM

LABORATORY UNIT

OPERATION BY ROYAL LAB

PALLIATIVE CARE UNIT

PROF. MOHAMED ABDELRAOOF MD
PROF. OF PAIN MANAGEMENT, CAIRO UNIVERSITY

الاسم : - يسر صالح

التاريخ:2026/10/4

المريضة تعاني من ورم غير حميد بالثدي الايسروقد

تم استئصال جزئي تحفظي

وتحتاج الي تلقي من البروتوكول العلاجي

Kisqali 200 mg –Femmara 2.5 mg-Zoladex 10.8

يستلزم تلقي العلاج الموجه والهرموني من العلاج لمدة عامان ثم يعاد التقييم

كما تحتاج الي تلقي 20 جلسة علاج اشعاعي متغير الشدة علي مدار 20 أسبوع

مدير المركز

د.مصطفي عباس

مركز علاج الأورام بيتا كير

للطوارئ 015 55 932 832

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فرع المهندسين : 53 ش جامعة الدول العربية - المهندسين - الجيزة
فرع مصر الجديدة : 60 ش خليفة المأمون - مصر الجديده
فرع الرحاب : المركز الطبي 1 عيادة رقم 212
فرع أسيوط : 1 شارع الجمهوريه بجوار المنفذ - برج الغول

Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

الهيئة القومية للتأمين الاجتماعي
مكتب خدمة المواطنين
عين شمس

بيان رقم تأميني

الرقم القومي	:	28107042500687
الرقم التأميني	:	14878122
الاسم بالكامل	:	يسرا صالح عبدالحكم رضوان
اسم الوالدة	:	انعام عبدالملاك
النوع	:	أنثى
تاريخ الميلاد	:	1981/07/04
محافظة الميلاد	:	اسيوط
قسم / مركز الميلاد	:	مركز القوصيه
الحالة التأمينية	:	ليس له بيانات تغطية أو معاش
بيان موجه إلى	:	لمن يهمه الامر

اعتماد مدير مكتب خدمة المواطنين

خاتم شعار الجمهورية

الخط الساخن
16217

تاريخ الطباعة : 2026/10/06 ساعة الطباعة: 10:18

Non UNIA Letter
[[DOCTYPE NON_UNIA_LETTER]]

Lab No.	740301	**Patient No.**	53320-7		
Name	الأستاذة / يسرا صالح عبدالحكيم			**Request Date**	14-09-2026 15:19
Date Of Birth	14-09-1983	**Sex**	Female	**Printing Date**	14-09-2026 15:23
Ref.By Prof.Dr.					

Hematology Report

Complete Blood Picture (CBC)

Test	Result		Unit	Ref.Range
RBC	3.0	↓	x10^6/uL	3.8 - 4.8
Hemoglobin (Hb)	9.0	↓	g/dL	12.0 - 16.0
HCT	27.0	↓	%	36.0 - 48.0
MCV	90.0		fl	80.0 - 97.0
MCH	30.0		pg	27.0 - 32.0
MCHC	33.3		%	32.0 - 36.0
PLT	250.0		x10^3/cmm	150.0 - 450.0
Total Leucocytic Count (TLC)	4.4		x10^3/cmm	4.0 - 11.0

Differential white cell Count

	Relative				Absolute			
	Result			Ref.Range	Result			Ref.Range
Neutrophils	60.0		%	40.0 - 80.0	2640.0		/uL	2000.0 - 7000.0
Lymphocytes	25.0		%	20.0 - 45.0	1100.0	↓	/uL	1500.0 - 4000.0
Monocytes	15.0	↑	%	1.0 - 10.0	660.0		/uL	200.0 - 1000.0
Eosinophils	0.0		%	Up to 6.0	0.0	↓	/uL	100 - 800
Basophils	0.0		%	Up to 1.0	0.0		/uL	Up to 100.0

Signature
Dr. Abdulwali Al-Ameri

Lab Test
[[DOCTYPE LAB_TEST]]

Visit Number:	71026504238	Client Name:	Q1 2026 Vip dr
Patient Name:	يسرا صالح عبدالحكم	Referred by Prof:	نيفين محمود احمد جابر
Age / Sex:	45 year / Female	Exam Date:	21 Feb 2026
Patient ID:	2140316365	Printed:	21 Feb 2026 10:14:10 PM

US

- **Pulmonary Valve:** Normal pulmonary morphology and flow.
- No evidence of intracardiac masses or thrombi.
- **Pericardium:** Normal thickness, No pericardial effusion.
- **Intact interatrial and interventricular septae** as seen by TTE.
- Normal thoracic aorta.

Conclusion:

- **Normal echo study.**

Best Regards,
Dr. Amr Yahia, MSc

Visit Number:	71026504238	Client Name:	Q1 2026 Vip dr
Patient Name:	يسرا صالح عبدالحكم	Referred by Prof:	تيفين محمود احمد جادو
Age / Sex:	45 year / Female	Exam Date:	21 Feb 2026
Patient ID:	2140316365	Printed:	21 Feb 2026 10:14:10 PM

US

Clinical data:	Check-up.

Transthoracic Echocardiography

LVED	4.1	(3.5 – 5.7 cm)	LA	3.3	(1.9 – 4.0 cm)
LVES	2.5	(2.2 – 4.0 cm)	Ao	2.8	(2.0 – 3.7 cm)
SWT	1.0	(0.7 – 1.1 cm)	RV	2.5	(2.5 – 4.1 cm)
PWT	0.8	(0.7 – 1.1 cm)	MVA	4.2	(2.0 – 6.0 cm^2)
FS	39	(25 – 45 %)	EF	70	(54-82%)

Comments:

- **Left ventricle:** Normal left ventricular internal dimensions with good overall global systolic function.
- Normal diastolic function.
- There are no segmental wall motions abnormalities can be detected at rest study.
- **Left atrium:** Normal internal dimensions.
- **Mitral valve:** Normal leaflets thickness and excursion. No mitral regurgitation.
- **Aortic valve:** Trileaflet valve, normal leaflets thickness and excursions. No significant systolic gradient is present across the valve.
- **Right sided chambers:** Normal internal dimensions with preserved RV systolic function.
- **Tricuspid valve:** Normal leaflets thickness and excursion, trivial tricuspid regurgitation.

دكتورة
شادية حسين مبروك
استاذ الباثولوجي
كلية طب عين شمس
عضو الاكاديمية الدولية للباثولوجي
دكتورة التحاليل الباثولوجية و الانسجة و الأورا

Date: 7/2/2026

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3 – 0.5 cm long.

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al carcinoma.

Specimen 2):

ns for E-cadherin

This confirming the

85%) of the
core=7.

c cells for PR. This is

Her2-neu. This is

ic cells for Ki-67.

cancer as determined

th:

Sh.

Shadia H. Mabrouk
١٣ شارع الخليفة المأمون -
العباسية - ص.ب ٣٠ -

د.عاصم البراشي
Dr. Asem Elbrashy
استشاري ومدرس جراحة الاورام والمناظير
دكتوراه جراحة الاورام جامعة القاهرة
عضو الجمعية المصرية لجراحة الاورام
استشاري وعضو هيئة التدريس
بالمعهد القومي للاورام - جامعة القاهرة

الاسم: سيدة / جميلة عبد الحكيم السن: التاريخ: / / 20

R/ مريضة / د.ج

45 y.o + ph. Presented w left breast cancer Post chemotherapy. wide local excision of the left breast mass w left axillary lymph nodes circumcision was done & sent for histopathology w showed mixed invasive ductal & lobular Carcinoma, Grade II w Tumor viability 30% & axillary lymph nodes mets. (3/20).

For your Assessment regarding Post-operative radio & chemotherapy.

الاستشاره والمتابعه بحجز مسبق
لحجز العمليات والاستفسارات
drassemelbrashy 010 3275 1816

r. Shadia H. Mabrouk, PH.D.
Professor Of Pathology
Ain Shams Faculty of Medicine
Membet of The International
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13 ElKhalifa El Maamoun St. Roxy.

بسم الله الرحمن الرحيم

دكتورة
شادية حسين مبروك
استاذ الباثولوجي
كلية طب عين شمس
عضو الاكاديمية الدولية للباثولوجي
دكتورة التحاليل الباثولوجية والانسجة والأورام

Name : *Yousra Saleh Abdel Hakam* *Age: 43 Y.* *Date:7/2/2026*

Referred by: *PROFESSOR DR. Nivin Gado*

Specimen : *Two specimens were received.*
Specimen (1): Tru cut biopsy from light suspicious left breast mass (BIRADS 5).
- *Two cores of greyish white tissue each is about 1.5 cm long.*

Specimen (2): Left axillary lymph node with focal cortical thickness.
- *Three small cores of greyish white tissue ranging from 0.3 – 0.5 cm long.*

MICROSCOPICALLY:

Sections examined from specimen (2) revealed three cores of fibrofatty tissue which are infiltrated by solid masses of neoplastic malignant cells of ductal carcinoma.

- No nodal tissue included in the tissue received.

DIAGNOSIS: Tru – cut biopsy from left axillary lymph node (Specimen 2):

- **Fibrofatty tissue with invasive duct carcinoma.**

Immunohistochemical staining was done on paraffin sections for E-cadherin ER, PR, Her2-neu, Ki-67 and revealed:

- **Positive** staining of the **neoplastic cells for E-cadherin. This confirming the diagnosis of ductal breast carcinoma.**
- **Positive** nuclear staining of moderate intensity in about (85%) of the neoplastic cells for **ER**. This is **consistent with Allred Score=7.**
- **Strong positive** nuclear staining of most of the neoplastic cells for **PR**. This is **consistent with Allred score=8.**
- **Negative** membranous staining of all neoplastic cells for **Her2-neu**. This is **consistent with score (0).**
- **Positive** nuclear staining in about **(20%)** of the neoplastic cells for **Ki-67.**
  - **According to the molecular subtype of this breast cancer as determined by gene expression profiling:**
  - **This immunohistochemical profile is consistent with:**
    - **Luminal A-like subtype.**

Sh.A

Prof. Dr. Shadia H. Mabrouk

١٣ شارع الخليفة المأمون - روكسي - امام متحف جمال عبد الناصر - فوق دجاج تكا ت: ٢٤١٧١٤٦١
العيادة : من ٣ - ٨ مساء عدا الجمعة

Dr. Shadia H. Mabrouk, PH.D.
Professor Of Pathology
Ain Shams Faculty of Medicine
Member of The International
Academy of Pathology
13 ElKhalifa El Maamoun St. Roxy.

بسم الله الرحمن الرحيم

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Specimen (2): Left axillary lymph node with focal cortical thickness.
- *Three small cores of greyish white tissue ranging from 0.3 – 0.5 cm long.*

MICROSCOPICALLY:

Sections examined from specimen (1) revealed two cores of fibrofatty tissue which are replaced and infiltrated by malignant epithelial glandular neoplasm of ductal carcinoma. The neoplasm is made up of small clusters, cords and trabeculae of large neoplastic cells. These displaying large hyperchromatic pleomorphic nuclei and scanty eosinophilic cytoplasm. Few large solid masses and occasional acinar formation are also detected. Few ducts which are colonized by neoplastic cells were identified "In situ carcinoma". The intervening stroma as well as the intratumoral tissue showing few foci of lymphocytes.

- No lymphovascular or perineural invasion was seen.

DIAGNOSIS:

Tru – cut biopsy from left breast mass (Specimen 1):

- **Poorly differentiated invasive duct carcinoma (Grade III) with:**
  - **Focal in situ component.**
  - **Focal intratumoral and stromal lymphocytes.**
  - **No lymphovascular or perineural invasion.**

Sh.A

Prof. Dr. Shadia H. Mabrouk

١٣ شارع الخليفة المأمون - روكسي - امام متحف جمال عبد الناصر - فوق دجاج تكا ت ٢٤١٧١٤٦١٠
المواعيد : من ٣ - ٨ مساء عدا الجمعة

Visit Number:	71026504238			Client Name:	Q1 2026 Vip dr
Patient Name:	Yousra Saleh Abd Elhakam			Referred by Prof:	Khaled Saud
Age / Sex:	44 year / Female			Exam Date:	22 Feb 2026
Patient ID:	2140316365	Acc	1816889	Printed:	23 Feb 2026 10:03:31 AM

PT

Side Findings:

- Bilateral maxillary sinusitis.
- Cardiomegaly for echocardiography correlation.
- Mildly increased FDG avidity along the right posterior scapular muscles (SUV max of 2.36), likely strain.
- Small hiatus hernia.
- Left cortical renal cyst (3x3.5 cm).
- Fluid filled endometrial cavity for ultrasound correlation.

IV) **The rest of PET / CT study appears normal.**

V) **Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.**

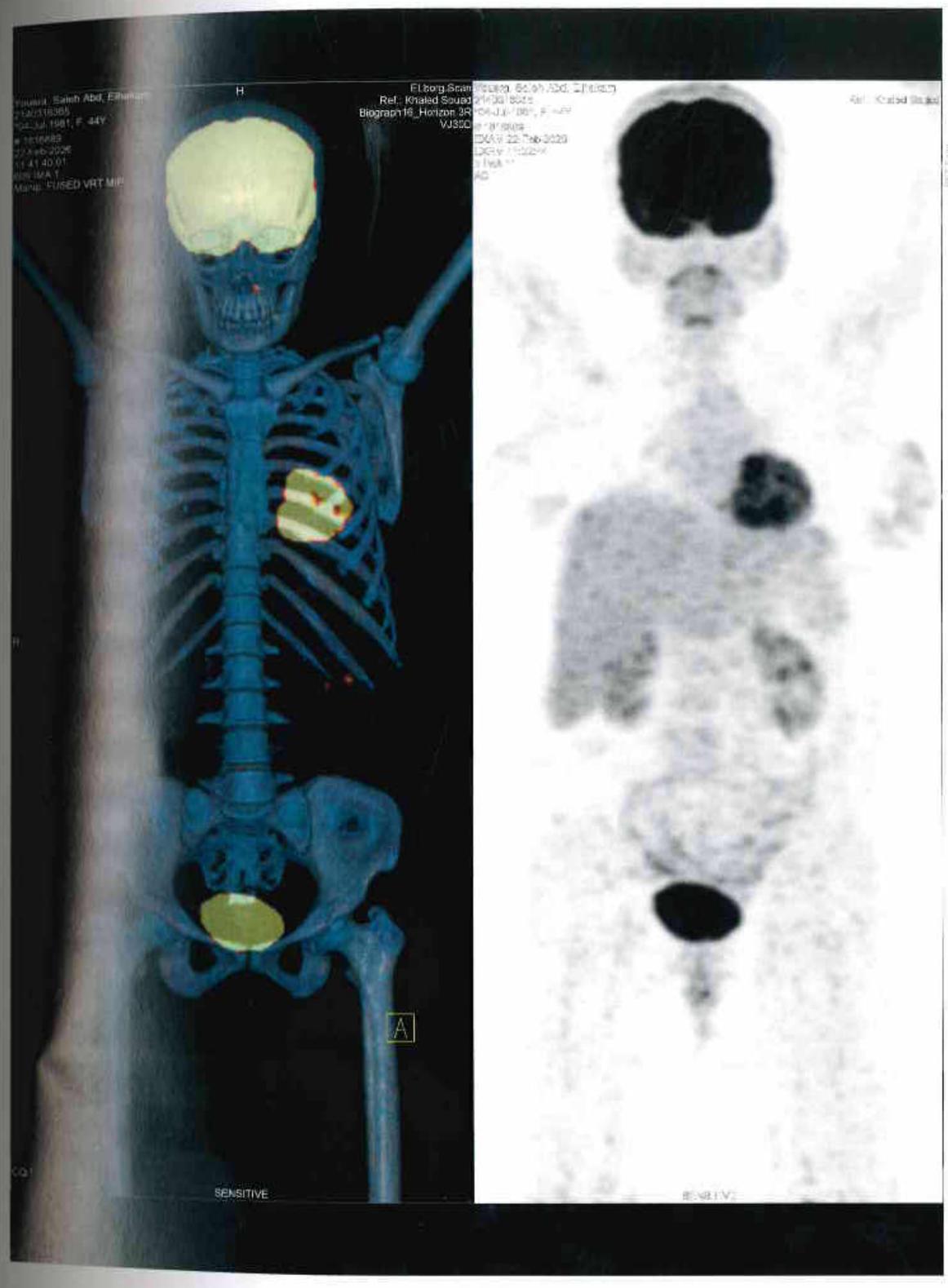
CONCLUSION:

PET-CT study for a reported case of left breast cancer revealed:

- **Heterogeneously dense left breast with UOQ low grade FDG avid area of architectural distortion with ill-defined soft tissue densities as well as foci of microcalcifiations associated with non FDG avid few left axillary and subpectoral lymph nodes, for SM correlation.**
- **Mild increase in bone marrow FDG uptake for CBC correlation.**
- **No other metabolically active sizable lesions elsewhere in body.**

Marina Essam, MSc

Prof.Khaled Aboulfotouh



Visit Number:	71026504238			Client Name:	Q1 2026 Vip dr
Patient Name:	Yousra Saleh Abd Elhakam			Referred by Prof:	Khaled Saud
Age / Sex:	44 year / Female			Exam Date:	22 Feb 2026
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PT

WHOLE BODY PET/CT EXAMINATION

History and Clinical Data:-

Patient data:-

- Female patient, 44 years old, Weight: 87 Kg, Height: 155 cm, Blood Glucose level: 130 mg/dl.

Radiopharmaceutical:-

- F-18-FDG 9 mCi injected I.V.

Clinical background:-

- Newly diagnosed case of left breast cancer.

Purpose of the study: -

- Initial assessment.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~2.02
- Liver showed max. SUV~2.78

Visit Number:	71026504238			Client Name:	Q1 2026 Vip dr
Patient Name:	Yousra Saleh Abd Elhakam			Referred by Prof:	Khaled Saud
Age / Sex:	44 year / Female			Exam Date:	22 Feb 2026
Patient ID:	2140316365	**Acc**	**1816889**	Printed:	23 Feb 2026 10:03:31 AM

PT

REPORT:-

Review of the PET, CT and the fused images revealed:-

I) Loco-regional (Both breasts and axillae):

a) Left side:

- Heterogeneously dense left breast showing a low grade FDG avid area of architectural distortion along its upper outer quadrant with ill-defined soft tissue densities as well as foci of microcalcifiations, it is seen reaching SUV max of 2.6, for sonomammography correlation.
- Non FDG avid few left axillary and subpectoral lymph nodes are noted, the largest seen measuring about 14.7x8.5 mm (showing fatty hilum yet mildly increased cortical thickness reaching 4.2 mm) for SM correlation.

b) Right side:

- Heterogeneously dense right breast yet no FDG avid sizable breast lesions.
- Non FDG avid few right axillary and subpectoral lymph nodes are noted, the largest seen measuring about 15x12 mm (showing preserved fatty hilum).

II) Liver:

- Enlarged liver with diffuse low fatty CT attenuation showing few areas of mildly increased FDG avidity with no corresponding definite sizable lesions for follow up.

III) The rest of the body:

- Average sized spleen with no metabolically active focal lesions.
- Non FDG avid few bilateral tiny pulmonary nodules (of average 3 mm).
- No FDG avid adrenal or pancreatic masses.
- Mild increase in bone marrow FDG uptake for CBC correlation.
- No metabolically active lytic/sclerotic osseous lesions.